POLICY NOTE · BUENOS AIRES PROVINCE · NOVEMBER 2026

Provincial transfers to municipalities do not follow need

Diego Lestani · Analysis of public data: INDEC Censo 2022 and Province of Buenos Aires open data (transfers 2025, health facilities 2025)

1. Need is concentrated where transfers per person are lowest. Greater Buenos Aires has a higher share of households with Unmet Basic Needs (NBI) than the rest of the province (7.0% vs 5.4%) but received about 190,000 pesos per person in 2025, against 348,000 elsewhere.

2. The formula rewards size and hospitals, not deprivation. Municipal size and hospital provision explain about 80% of the variation in transfers per person; once they are accounted for, deprivation has no significant association with transfers.

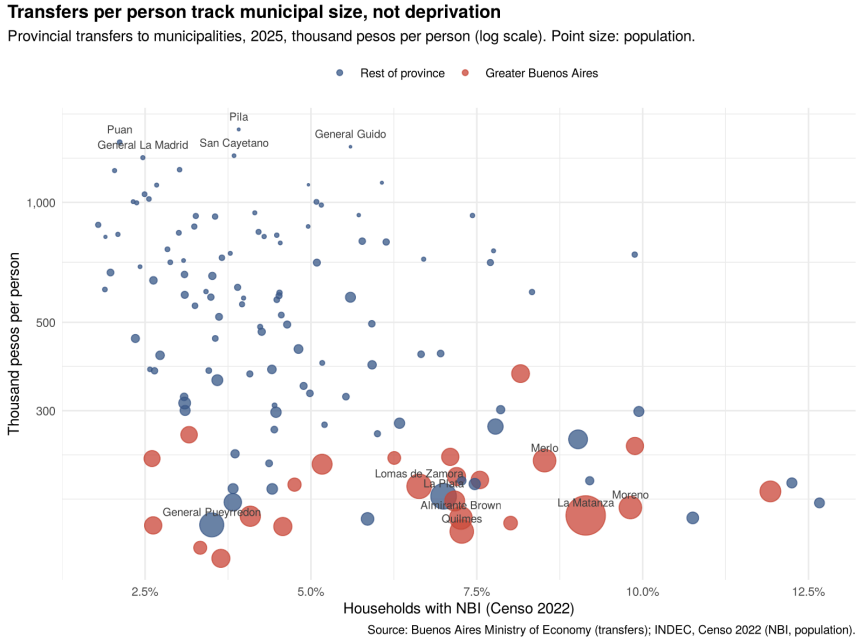
3. Municipal averages hide most of the problem. 83% of the variation in deprivation across census tracts lies *within* municipalities. Sewer connection ranges from 87% of households in the least deprived fifth of Greater Buenos Aires tracts to 28% in the most deprived.

THE ISSUE

Buenos Aires municipalities receive a fixed share of provincial revenue through coparticipación (Ley 10.559) plus smaller earmarked funds: about 4.35 trillion pesos in 2025, 77% of it coparticipación. Each municipality's share is set by one coefficient: 58% reflects population, area and inverse tax capacity; 37% health indicators (beds, consultations, patient-days, discharges, inpatient facilities); 5% social services. No component measures deprivation. This note asks whether the allocation reaches the places with the greatest need.

WHAT THE DATA SHOW

Transfers are concentrated among the less deprived municipalities. Ranking the 135 municipalities from most to least deprived, households with NBI have a concentration coefficient of −0.18 (concentrated among the most deprived), while transfers have +0.11 (concentrated among the least deprived). The pattern holds, more weakly, with an income-closer measure of deprivation (INDEC's IPMH).



Transfers per person (2025, log scale) against the share of households with NBI (2022), by municipality. Larger, more deprived municipalities in Greater Buenos Aires (red) receive the least per person.

Size and hospitals drive the allocation. A municipality 1% larger receives about 0.3–0.4% less per person; size alone explains three quarters of the variation. Municipalities with more public hospitals per resident receive more, as the health com-

ponent intends. Neither factor tracks deprivation: small rural municipalities with their own hospitals receive the most per person, large deprived peri-urban ones the least.

Services lag where deprivation is concentrated. Primary care centres are well located, but public general hospitals are not: in Greater Buenos Aires, 10.8% of people in the most deprived fifth of census tracts live more than 5 km from one, against 1.5% in the least deprived. The sewer gap is steeper still, and transfers bear no relation to it.

Indicator	Greater Buenos Aires	Rest of province
Households with NBI (2022)	7.0%	5.4%
Provincial transfers per person (2025)	≈ 190,000 pesos	≈ 348,000 pesos
Sewer connection, least vs most deprived fifth of tracts	87% vs 28%	82% vs 23%
Distance to nearest public general hospital, least vs most deprived fifth	1.8 vs 2.9 km	3.5 vs 3.8 km

The gap is large. As an illustration only, splitting the same pool 50% by population and 50% by households with NBI would raise transfers by more than 20% in 24 municipalities, mostly in Greater Buenos Aires, and cut them by more than 20% in 92. The exercise ignores the fixed costs of small municipalities; it shows distance from need, not a feasible reform.

POLICY IMPLICATIONS

1. **Introduce an explicit need component, phased in.** Allocate a share of the general regime on census-based deprivation (NBI or IPMH, updated with each census), with hold-harmless floors for small municipalities during the transition.
2. **Reconsider what the health component rewards.** Paying for beds and activity favours municipalities that already run hospitals; weighting part of it by population served, or coverage of deprived catchment areas, would align it with need.
3. **Target infrastructure below the municipal level.** Most deficits lie within municipalities, beyond the reach of formula transfers. A provincial water and sewer fund could allocate projects on tract-level census deficits, which are already available.
4. **Publish the components of the coefficient.** The province computes each coefficient from its components but publishes only the total; releasing the breakdown would allow the formula to be assessed and reforms simulated.

Trade-offs: small municipalities have real fixed costs; changing Ley 10.559 requires legislative approval and touches every municipal budget; and need-based transfers can weaken incentives to reduce measured deprivation, which favours census indicators municipalities cannot manipulate.

SCOPE AND LIMITATIONS

The analysis is descriptive: regressions describe associations, not causal effects. NBI measures structural deprivation (housing, sanitation, schooling, subsistence capacity), not income; results are checked with IPMH. Deprivation and population are from the 2022 census; transfers and facilities from 2025. Distances are straight-line. Full methods, code and data: github.com/diegolestani/pba-transfers-need; interactive map: diegolestani.github.io/pba-transfers-need. Formula weights as described in CREEBBA, *Indicadores de Actividad Económica* 161, and Ley 10.559.